



OFFICE OF THE POLICE COMMISSIONER

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June 5, 2009

Memorandum for: Deputy Commissioner Trials

Re: **Police Officer Delvia Moore**
Tax Registry No. 918682
28 Precinct
Disciplinary Case No. 83467/07

CHAN

The above named member of the service appeared before Assistant Deputy Commissioner David S. Weisel on September 24, 2008 and was charged with the following:

DISCIPLINARY CASE NO. 83467/07

1. Said Police Officer Delvia Moore, assigned to the 28th Precinct, on or about January 30, 2007, while on sick report and assigned to the Manhattan Court Section, was wrongfully and without just cause absent from her residence without the permission or authority of said Officer's District Surgeon and/or Medical Division Sick Desk Supervisor from 1600 hours to 1825 hours.

PG 205-01, Page 2, Paragraph 4 REPORTING SICK/PERSONNEL MATTERS

2. Said Police Officer Delvia Moore, assigned to the 28th Precinct, on or about January 17, 2007, while assigned to the Manhattan Court Section, made false or misleading statements to Medical District personnel exaggerating the extent of her injuries, to wit: Officer Moore told Medical District personnel, in sum and substance, that she could not perform in a restricted duty capacity, and appeared before the medical District using a cane, which, in light of observations made of Officer Moore, are in fact false.

P.G. 203-10, Page 1, Paragraph 5

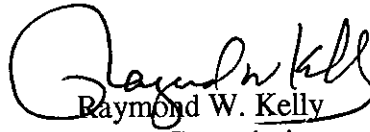
PROHIBITED CONDUCT

In a Memorandum dated December 9, 2008, Assistant Deputy Commissioner Weisel found the Respondent GUILTY of both Specifications. Having read the Memorandum and analyzed the facts of this instant matter, I approve the findings, but disapprove the penalty.

The Respondent's deceptive behavior, combined with her misleading statements, warrants a greater penalty than that which has been recommended. Such egregious misconduct cannot and will not be tolerated. Therefore, the penalty in this matter shall consist of the Respondent being immediately suspended without pay for 30 days and that she be DISMISSED from the New York City Police Department; however, this penalty of dismissal will be held in abeyance pursuant to Section 14-115 (d) of the NYC Administrative Code for a period of one year, during which time the Respondent will

remain on the force at the Police Commissioner's discretion and may be terminated at any time without a further hearing.

Prior to the expiration of the thirtieth suspension day, or prior to any proposed change in the Respondent's duty status, this office is to be immediately notified.


Raymond W. Kelly
Police Commissioner



POLICE DEPARTMENT

December 9, 2008

MEMORANDUM FOR: Police Commissioner

Re: Police Officer Delvia Moore
Tax Registry No. 918682
28 Precinct
Disciplinary Case No. 83467/07

The above-named member of the Department appeared before me on September 24, 2008, charged with the following:

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PG 205-01 Page 2, Paragraph 4 – REPORTING SICK/ PERSONNEL MATTERS

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PG 203-10 Page 1, Paragraph 5 – PROHIBITED CONDUCT

The Department was represented by Daniel Maurer, Esq., Department Advocate's Office, and the Respondent was represented by Michael Martinez, Esq.

The Respondent, through her counsel, entered a plea of Not Guilty to the subject charges. A stenographic transcript of the trial record has been prepared and is available for the Police Commissioner's review.

COURTESY • PROFESSIONALISM • RESPECT

DECISION

The Respondent is found Guilty as charged.

SUMMARY OF EVIDENCE PRESENTED

The Department's Case

The Department called Doctor David Lichtenstein, Doctor Brian Mignola, and Sergeant Michael Hengel as its witnesses.

Doctor David Lichtenstein

Lichtenstein was a Department physician specializing in internal medicine. He also examined patients for conditions other than those related to internal medicine.

Lichtenstein examined the Respondent on March 22, 2007. He had not met her before. She presented with pain and discomfort in the left knee after a hyperextension. Lichtenstein explained that in contrast to flexion, which is bending the knee, hyperextension occurs when the knee or leg comes forward too quickly. The term "hyperextension" was the Respondent's "own words."

Lichtenstein testified that he performed a gross examination, meaning he compared the knees to each other. There was no gross deformity of the left knee compared to the right knee. On cross-examination, counsel asked about pain to the knee caused by "a tear to the medial meniscal."¹ This referred to one of the supporting ligaments of the knee. Lichtenstein explained that "usually, and once again, there's no absolutes," there would be a gross deformity of the knee after "an acute tear or a chronic subacute tear causing pain." Fluid would enter the joint space and then into the skin. The affected knee would appear two or three times the size of the unaffected knee.

¹ Defense counsel appeared to be reading from a document as he asked this question.

Lichtenstein testified that the Respondent told him she had suffered from osteoarthritis since February 2007. Osteoarthritis, the doctor explained, was bone grinding against bone with the absence of cartilage. The Respondent came into the Medical Division office limping and using a cane for assistance. She said that she could walk only about half a block before experiencing "extreme pain." Lichtenstein testified that a person who had three surgeries on the knee would have some degree of osteoarthritis. However, in Lichtenstein's experience, with a hyperextensive re-injury to an osteoarthritic knee, there are usually gross physical findings "that would lead you to believe that there is an acute injury that would incapacitate" the member, or prevent her from resuming duty. Here, because the Respondent "was in obvious pain, limping with the assistance of a cane," but without gross swelling or instability of the knee, "there was a question." He testified that he was only determining her duty status, and he found that she was ill on that day.

Lichtenstein kept the Respondent out sick until March 26, 2007, at 0800 hours. He then placed her on restricted duty.

Lichtenstein could not say that he had suspicions regarding the Respondent because he did not know enough about her previous injury. He testified that while he gave the Respondent "the benefit of the doubt" by giving some more days to recuperate from the hyperextension, he also made a notification to Absence Control (i.e., the Absence Control and Investigations Unit [ACIU]). Lichtenstein testified that it was his custom to do so with members who were on restricted status in the Special Medical District (SMD), and who complained of a physical injury with no gross medical findings. He added that it was his custom to contact Lieutenant Valenti, his supervisor, and inform him of the "discordancy" between the complaint and the findings. Lichtenstein testified that it was within his judgment whether to make notifications to ACIU if a member told him "something that was inconsistent" with what he could independently verify.

Usually, these notifications are by informal conversation, sometimes “along the lines of . . . doctor feels MOS may be exaggerating extent of injury.” He testified that he also makes notifications to ACIU “for the specific purpose” of helping members receive their disability or pension payments “in a timely fashion.” Lichtenstein testified that in his experience, the notification helps the member the majority of the time.

On direct examination, Lichtenstein testified that the Respondent was in the SMD because, by the Patrol Guide, a member is placed there and subjected to “slightly more” scrutiny by the Medical Division if the member reaches a certain level of sick days. On cross-examination, however, Lichtenstein said that he “made it a point not to equate those persons assigned to Special Medical District to a heightened level of scrutiny.” He nevertheless acknowledged, again, that assignment to the SMD meant that “a determination had already been made to . . . keep a closer eye on that individual.” He had no “independent reason” to doubt what the Respondent was telling him.

Lichtenstein testified that pain is a subjective complaint, whereas a swollen knee is objective. “If the objective findings do not add up to the subjective complaints, there may be a question.”

Lichtenstein was unaware at the time of his examination that the Respondent had two prior surgeries to the knee in question. One could have “surgery two years ago, and make a total recovery . . . depending on the nature of the illness and the type of surgery.” He did not think it was “germane” that she had, since his examination, a third surgery on the knee.

Lichtenstein testified that if someone told him she could walk half a block without pain, it did not necessarily mean that she could not walk at all after half a block. It was possible that the patient could walk with much discomfort, would have to rest for a few minutes, or could use some kind of aid to walk.

Lichtenstein testified that if someone is experiencing pain to the knee, he would want to know about it, even if the patient said, "look, it hurts, but I can make it, I can still walk even though it hurts."

Lichtenstein stated that it was possible that someone who had surgery to the knee might not feel pain in a sitting position with the leg elevated. He testified that usually, patients with osteoarthritis have pain right after they get up from a sitting position. The joint is "cold," but when the patient moves, "especially over a one- or two-block period, the pain actually lessens as the blood flows over the joint and warms them." Usually, the most difficult thing for a patient with severe osteoarthritis and chronic leg injury is to get up and walk. This was "kind of pretty much always the case."

Counsel for the Respondent, on cross-examination, asked Lichtenstein about a "Grade 2 to 3 degenerative joint disease finding." The doctor testified that most individuals who have engaged in sports will, by the age of thirty or forty, have some degenerative joint disease. Many "athletes" have Grade 2 to 3 tears in the medial or lateral meniscus, and while it will cause some discomfort and swelling of the joints, "it doesn't impede their activities" or "necessarily mean that your knee is unstable." However, "a chronic degenerative process like that eventually gets to the point where the knee joint becomes unstable, where the pain becomes intolerable even on slight movement." The fluid accumulation in the knee causes an inability for the patient to even flex the knee.

Lichtenstein testified that "invariably," the initial orthopedic surgery examination "is relatively unremarkable in terms of motor strengths and range of motion." Only acute tears to the anterior cruciate ligament (ACL), acute fractures, or "gross fluid build-up in the knee" would lead to "an abnormal physical finding."

Lichtenstein testified that nothing was necessarily indicated by “extensive synovium . . . in the medial lateral and patella femoral notch area.” Synovium consists of “skin between the cartilage.” Individuals who engage in “high impact aerobics,” like runners or football players, usually have thickened synovium due to “chronic microtrauma” in the joint space.

Doctor Brian Mignola

Mignola was a Department physician, and also had a family practice on Staten Island. He had been a physician for approximately 15 to 16 years. He had experience with knee injuries and pain management. He was not a surgeon by practice, as opposed to his Department title. He had assisted on various surgeries, including knee arthroscopies, appendectomies, bypasses, and caesarean sections. He testified using the Respondent’s “medical file,” which consisted of the “Department notes” and the operative notes from the first surgery, written by the Respondent’s private physician.

Mignola testified that the Respondent’s original injury was a medial lateral meniscal tear and degenerative joint of the left knee, and inflammation of the synovium surrounding the knee. The meniscus, Mignola explained, was cartilage. Her surgeries were partial meniscectomies, in which damaged parts of the meniscus are removed in the hope that new cartilage will grow and renew the “contour,” and reduce the inflammation.

Mignola did not speak with the Respondent’s private physician. The Department had a form that the treating physician was supposed to submit, and Mignola received a “summary statement” from this doctor. Mignola later referred to these documents as “Treating Physician Summary Statements.” The private doctor usually “documents the nature of the injury, the prognosis, the treatment plan and a rough idea” of when the doctor feels the member might

return to limited or full duty. The private physician's diagnosis here was "internal derangement" of the left knee. She had two surgeries on the knee "up until the time [he] saw her."

The Respondent's first visit to Mignola was on October 22, 2006. She complained of buckling in her left knee after walking on stairs, as well as "weakness." She also stated that she was "pre-op" for surgery on that knee.

Mignola testified that the Respondent had surgery on that knee on November 3, 2006. She would have had arthroscopic surgery, where the meniscus would be shaved down. Mignola testified that the patient's time to return to work depended on the profession involved, but he had "many patients" that returned within one week. This was not a "rule of thumb." The average time was about one month.

Mignola next examined the Respondent on November 13, 2006. The knee was immobilized by, Mignola believed, a brace, and she was ambulating with a cane.

Mignola next saw the Respondent on December 1, 2006. He recommended that her duty be continued as "sick." On December 15, 2006, Mignola saw the Respondent for a post-operative follow-up visit. She was still ambulating with a cane, and complained of pain and discomfort. She had a slight "gait" or limp while walking. Mignola made another follow-up appointment for her, and wrote a notation to evaluate her for limited duty status.

On December 26, 2006, Mignola testified, he examined the Respondent again. She "still had restrictions in range of motion, tenderness," and a limp. "At that point," Mignola felt that an evaluation by a Department orthopedist was in order. He believed an appointment was scheduled but "got pushed back a little bit." At his next visit with the Respondent, on January 10, 2007, she had not yet been seen by the orthopedist. Mignola continued her sick status and scheduled a follow-up visit in one week.

On January 17, 2007, Mignola noted that the Respondent still had tenderness, but was negative for crepitus, which he described as “that sandpaper feeling.” Once again, Mignola “felt she had an orthopedic evaluation that was due within the week and I deferred to the Department orthopedist at that point.”

Mignola indicated that during the January 17, 2007, visit, the Respondent “complained of ongoing stiffness and pain in the knee, and she was unable to ambulate or sit for any prolonged period of time.” Mignola testified that a patient with the Respondent’s condition could experience pain when the knee is in a flexed position, i.e., when the person is sitting down. Standing up or stretching out the knee could alleviate the pain, but standing or running can also cause pain.

Mignola testified that the reason he “pushed for” the orthopedic evaluation was that “from the time of surgery to this point the duration was extended more than what I’ve usually seen in the past.” He felt that her limp was “slightly” exaggerated compared to the extent of her injury. She had been out sick for about four months, the period between the first visit in October 2006 and this most recent visit on January 17, 2007.

After this visit, Mignola testified, he “wasn’t sure where this was progressing, whether or not truly there was an issue.” He “wasn’t sure to the degree of her injury – more of an instinct,” and “wanted to get a better picture of what was going on and what the next step was as far as [ACIU] was concerned.” He had a Police Administrative Aide call the orthopedic clinic to expedite the Respondent’s appointment, and contacted Valenti at ACIU.

Mignola believed he “made it known” to the Respondent that he was “considering” placing her “back” on restricted duty “at some point.” He did not specifically remember her response, but “there was some resistance based on the level of pain she was experiencing.” There was “no documentation” that she “ever refused to go back to limited duty.” He recalled

that when he brought up the possibility of the Respondent returning to limited duty, she “wasn’t thrilled” and “seemed to resist it.”

Mignola testified that he felt “she should be better by now” to return to restricted duty. “On the average,” based on the time of surgery to the January examination, she should “have been back to a sedentary work status.”

Mignola stated that the Respondent was next examined on January 24, 2007. She was still ambulating with the cane, and had some swelling and restriction of motion. The swelling could have been consistent with pain. Mignola testified that his intent was to keep the Respondent out sick until the orthopedic evaluation, and defer to the orthopedist’s evaluation of the Respondent’s duty status.

Based on the Respondent’s medical file, Mignola testified that the Respondent was seen by Dr. Goldman, the Department orthopedist, on January 28, 2007.² Goldman recommended that the Respondent be placed on restricted duty. The Respondent was also examined on March 22, 2007. She was actually placed on restricted duty in March 2007. On May 31, 2007, she had a follow-up orthopedic evaluation with Dr. Murray, who recommended her for restricted duty, but characterized her prognosis as “good for full duty in approximately three months.”

Mignola testified that several personnel could return an officer to full duty: the District Surgeon, or, for example, the Department cardiologist if it were a cardiological case.

Mignola testified that the healing time for “someone with swelling who’s complaining of pain with their history of knee ailments” varied depending on “the patient, the activity” and medication, but was generally “within several days.” A person with “this particular knee ailment and this particular history of pain and surgical procedures” could have chronic or intermittent pain, depending on the damage to the knee. It would be “particularly painful” to lift heavy

² On cross-examination, Mignola testified that he referred the Respondent “first to the orthopedist and then I referred her to Dr. Goldman.”

objects. Such a person would try to compensate, possibly by putting more stress on the other knee. Most people do this without even realizing it. This is why people limp.

Mignola testified that the "constant flexing of the knee" would lead to "discomfort" while driving, but agreed that most people, when driving automatic-transmission vehicles, only use their right foot. The patient could "have a problem getting into a vehicle which is particularly high off the ground."

On cross-examination, Mignola agreed that he did not perform an "extensive" examination of the Respondent. He concurred that it was important for a patient and doctor to be honest with each other. He kept the Respondent out sick each time he examined her, and never changed her duty status. He considered it "unusual" to learn that she was out sick for five months after her first arthroscopy. There was no notation in the Respondent's medical file indicating that she had applied for disability retirement.

Mignola agreed that the lack of visible swelling did not mean that the patient was not in pain. Pain is subjective, and some knee injuries "could result in a person feeling pain after using the knee for a prolonged period of time." There are also injuries where the pain starts right after the knee begins to move, then gets better. He agreed that a person's amount and tolerance of pain determined how ambulatory she was.

Mignola agreed that someone who has "discomfort" in one knee might use a cane to shift the burden away from the affected knee. Not everyone who uses a cane is completely disabled. The cane also prevents future injury "with the leg giving out." A shopping cart may also be useful for this purpose. While Mignola agreed that in some cases, a person will state that she can walk about a block or block-and-half "and then my knee starts to throb," he would not necessarily recommend a cane because the person will "crutch on the cane," meaning she will use it more than she should.

Mignola testified that physical therapy is considered first, before surgery is indicated. Three surgeries was more than average, in his opinion. The idea that the Respondent was “the type of person to undergo a needless surgery . . . never came into play.” Unless private insurance is used, a police officer must receive authorization from the Department for surgery.

Mignola testified that it was the exception, not the rule, for the Department physician to speak to the private physician, rather than simply to rely on the private doctor’s report and the second opinion from the Department orthopedist. This was the case in workers’ compensation and no-fault insurance matters as well. Mignola agreed that if there was “a significant conflict” between what he found and what the private physician claimed, then “there potentially would be a one-on-one” consultation.

Mignola could not state “scientifically” why he referred the Respondent to ACIU. He testified that medicine “can be as much an art as a science. There was something that struck me that was not right, instinct; but I wanted to give her the benefit of the doubt.” Thus, he referred her both to ACIU, and to the Department orthopedist for an objective examination.

On re-direct examination, Mignola agreed that the Respondent “would be able to sit at a desk and perform clerical functions and be able to stretch or flex her left knee to relieve pain.” Mignola added that he was authorized, as part of the return-to-duty process, to place parameters on a member’s activities, like “no stairs, no patrol, no lifting, allowed to change positions as needed.” Mignola intended the Respondent to do “just basic desk work – answering the phone and nothing more than that.”

“For the most part,” Mignola testified, if someone said she could not walk more than half a block without pain, he would expect her to be using “a cane or a walking device all the time.” He would expect that “someone with this particular medical history” would be using her cane “most of the time” if she used it at all as a walking aid.

Sergeant Michael Hengel

Hengel was assigned to the Medical Division. Beginning on January 17, 2007, he investigated an allegation that the Respondent was “presenting herself with crutches or a cane,” and possibly was exaggerating the extent of her injuries. Mignola stated that the Respondent “was coming to the clinic all the time with the cane,” and had told Mignola “that she could not do restricted duty.” Mignola made the referral by speaking to Valenti, the Commanding Officer of ACIU. The statements attributed to the Respondent by Hengel were “on the referral that Lieutenant Valenti took from Dr. Mignola.”

On direct examination, Hengel testified that he made several observations in the area of the Respondent’s residence, which was located on Croes Avenue in the Bronx. On two occasions, he videotaped these observations (see Department’s Exhibit [DX] 1).³ On cross-examination, Hengel admitted that he went to make observations of the Respondent on five dates, but only actually saw her on two: January 30, 2007, and March 2, 2007.

The tape was played in the Trial Room, and Hengel testified about what it was portraying. The first part of the tape was made at 2:02 p.m. on January 30, 2007. This date, Hengel testified, was “about two weeks after the doctor had made a referral.” This was not the first time he went to make an observation, but it was the first time he actually saw the Respondent. The tape showed the Respondent putting her children into her vehicle, a Chevrolet Suburban SUV, which was parked in the back of her apartment complex. The tape showed her stepping on the SUV’s running board.

The tape next displayed an observation at approximately 6:28 p.m. on January 30, 2007. The camera’s night vision feature was turned on. Hengel stated that she was “was arriving back,” and parked behind her building. The Respondent got out of the vehicle and went to the

³ The tape was marked “PO Alexander.” The Advocate indicated that Alexander was the Respondent’s maiden name.

back. She then went to the passenger side of the vehicle. She removed one of her children from the rear of the vehicle and placed the child in the stroller.

Hengel testified that the Respondent was out past the end of that day's pass hours, which were 12:00 p.m. to 4:00 p.m. Hengel asserted that she did not "have permission or authority to be outside of those hours."

The tape next displayed the Respondent on March 2, 2007, at around 1:19 p.m., outside a doctor's office on Westchester Avenue in the Bronx. The Respondent left the medical office and returned to her residence, where she picked up another female. At 1:40 p.m., the Respondent arrived, in the same SUV, at the Bronx Health Care Unit. This unit was located within the 50 Precinct in Riverdale. The Respondent got out of the driver's side of the vehicle, carrying a cane in her right hand. She was not holding this cane, Hengel said, when she left the doctor's office on Westchester Avenue. She and "another unknown female" walked to the precinct. Hengel asserted that the Respondent's limp became more pronounced as she got closer to the precinct.

At 3:33 p.m., the tape displayed the Respondent leaving the precinct and walking back to her SUV. At 3:54 p.m., after leaving the precinct, she drove back to her residence on Croes Avenue. The Respondent and the second female walked in the parking lot. The Respondent was not holding the cane, and in Hengel's opinion, "seem[ed] to be walking fairly normally."

On cross-examination, Hengel stated that the sole observation he made in February 2007 was on February 6th. He was not aware of any circumstance in which someone told the Respondent, "no, no, you're going back to restricted duty and she refused." Hengel agreed that he probably would know if something like that had occurred. He also agreed that when he needed to move his car, he shut off the camera. Nonetheless, Hengel testified that there "were no interactions that [he] saw that weren't memorialized to that videotape."

The Respondent's Case

The Respondent testified on her own behalf.

The Respondent

The Respondent had been a member of the Department for over eleven years. On October 15, 2004, she received a line-of-duty injury. She was working the midnight tour, and responded to a noise complaint around 3:00 or 4:00 a.m. The perpetrator pushed the Respondent, and her partner grabbed the perpetrator. The perpetrator then grabbed her partner's gun and "put it in my face." The Respondent grabbed the gun, and all three fell to the ground and fought for the gun. The Respondent banged her knee on the concrete. The next day, her leg swelled up, and she "wound up tearing my meniscus and everything."

The Respondent testified that she had arthroscopic surgery on February 4, 2005. She was out sick between the injury and the surgery. The surgery was for torn medial and lateral meniscus, and torn ligaments. She had a private physician, Dr. Kushner. The Respondent saw Kushner every two weeks, she believed, prior to the surgery.

After surgery, the Respondent stated, she had physical therapy, which consisted of attempts to "stimulate the knee," using hot and cold pads, trying to return flexibility to the knee, and trying to walk with a cane or crutch, although she was using a cane "during that time period." She was in "a lot" of pain. At first, she was driven to her appointments, then started driving herself. She was on crutches at first for about a month, then a cane. She was instructed not to put a lot of weight on the knee.

The Respondent testified that between the time of the injury and the first surgery, the Department did not try to place her back on duty, and never challenged her as to whether she was really injured.

The Respondent stated that she was out sick for four to five months after the February 2005 surgery. She returned to duty in June or July of 2005. She did not “fight going back to restricted duty” or disobey any order to return to duty. While on restricted duty, she worked at the Bronx Court Section. She asserted that her knee would swell up and “was buckling somewhat, and it was just constant pain.” She had the knee drained due to swelling. She was taking Vicodin, Percocet and Oxycontin at various times. No one from the Department challenged as to whether she needed those pain medications.

The Respondent was on maternity leave between September 28, 2005, and June 16, 2006. She was on bed rest because of her knee and the fact that she was pregnant with twins, leading to a high-risk pregnancy.

The Respondent asserted that the knee did not improve. Her doctors told her that her “options” were to have another arthroscopic surgery. The Department authorized the second surgery, which took place on November 3, 2006. The doctors needed to remove the pins they had put in. The knee did not heal properly, and the meniscus had re-torn. The doctors were hoping the knee would heal, and told the Respondent to “try the therapy again to see what happens from there.” During this time, the Respondent was visiting her private orthopedist every two weeks, then every three weeks. She also did physical therapy again as well as home exercises.

After the second surgery, the Respondent stated that at first, her knee was immobilized and she ambulated with crutches. When the immobilizer was removed, she used a cane. She stated, however, that she did not want “to also use the cane.” She would use it when necessary, such as when she stood for extended periods, or would walk long distances. If she knew she was going to be doing these kinds of activities, she would take the cane with her. If she had been seated without pain, she could walk about half a block before feeling pain. She would keep

walking, but with pain. She could still drive because she only used her right foot on the pedals, and could extend or flex her left leg to "get some movement" with the left knee.

The Respondent stated that after the second surgery, she reported to the Medical Division every two weeks. The Department's examinations were "nothing extensive." The doctor would compare the knees to see whether there was swelling. One of the Department examinations was on January 17, 2007. The Respondent recalled meeting Mignola for probably fifteen minutes. She claimed, however, that he did not examine her. She asserted that normally, she would give the Department physician her "doctor's notes, because I had to see the doctor prior to seeing him." Mignola asked "what the doctor said," and how the Respondent was feeling. She told him she was in a lot of pain and had done "some prolonged standing recently" relating to her father's death on January 9, 2007. She had also done more walking. The Respondent testified that she "would have explained" to Mignola what she "was able to do with respect to walking and distances and things."

The Respondent asserted that when she was on restricted duty in the Bronx Court Section, she did a lot of paperwork. The Department offices and the District Attorney's offices were located in separate areas. She took an escalator and an elevator to get around, and "had to do a lot of walking." In March 2007, she was walking with a cane. The Respondent said that the problem in the Bronx Court Section was the walking; she did not have to take stairs.

The Respondent testified that on January 17, 2007, she told the District Surgeon that she did not think she would be able to perform restricted duty at that time. He asked her "if I feel I would be able to do it; I said, no. . . . I don't feel like I would be able to do it." On direct examination, she testified that she answered the doctor in this way because

I know I wouldn't just be sitting there answering phones. My job details me doing a lot of walking, and I know I was in a lot of pain. Even pushing a stroller, doing stuff with

the kids, I know I'm in pain with that. I know it's constant pain and I was doing a lot of walking at work.

On cross examination, however, the Respondent stated that she walked "a lot" when doing restricted duty, and was in pain, but "not constant all the time."

The Respondent asserted that she "knew what it was like to work restricted duty," and honestly believed, when she talked to the District Surgeon on January 17, 2007, that she would not be able to perform restricted duty.

The Respondent testified that Mignola told her that, in the future, she likely was going to be put back on restricted duty. She denied that she was "ever" ordered back to restricted duty, or that she refused to go back onto restricted status. She stated, however, that she was placed on restricted duty in March 2007. She said that the doctor told her that "they were returning me to work." She asserted that she did not "fight them in any way," and appeared for work as ordered. She stated that she did not "disobey anybody."

The Respondent testified that her knee did not improve going forward from March 2007, and she continued to see both her private physician and the Department physician.

The Respondent testified that she was currently out sick, most recently as of May 2008. The most recent of three surgeries to her left knee was on July 11, 2008. This was due to a hyperextension and buckling of the knee that occurred in May 2008. She was arriving home, when her knee buckled as she got out of her car, and she fell. She stated that she "had finished work" at this time.

The Respondent testified that she went to the emergency room, where X-rays were taken. She did not go out sick, however, and went back to work with crutches. A week later, her knee buckled again, and she returned both to the hospital and the Department doctor. The Department doctor recommended an MRI (magnetic resonance imagery). The MRI was taken in May or

June of 2008, and it revealed a re-tear. The Department designated this as “old line-of-duty because it was because of the past surgery.” The Respondent testified that she was kept out sick by the District Surgeon after the second fall.

The Respondent testified that the Department wanted to see her MRI and X-ray films, and that her private doctor was “in the loop on this.” Her private doctor recommended a third surgery, which the Department authorized, although one doctor at the Medical Division mentioned that there was a 50% chance that the third surgery would not help her. The Respondent had the third surgery on July 11, 2008. She stated that this surgery was “more intense than the meniscus,” and involved removing a part of the bone. She currently saw the District Surgeon every two weeks. As yet, there had been no discussion of returning to restricted duty. She was not able to put full weight on the knee.

The Respondent testified that her prognosis was to use a brace that would take weight away from her meniscus. If that did not work, the doctors would need to remove some of the tibia. She testified that the doctors told her that eventually, she would, or at least might, need a total knee replacement, but she was “not old enough to warrant that yet.” On cross-examination, the Respondent stated that her doctors had removed the “immobilizer” that was on her knee. In response to the Department’s question of whether she was using “a soft brace or anything like that,” she testified that she was waiting for the Medical Division to approve the use of a “brace.”

The Respondent testified that after her July 2008 surgery, she was using crutches. She had people drive her to appointments and go shopping for her.

The Respondent believed that on January 30, 2007, shown on the videotape (DX-1), she was going to the doctor and getting into the vehicle. She was not using her cane because the distance between her residence and her vehicle was “not even half a block that I have to walk to it.” The building in the background was her residence, an apartment building. Her cane was

collapsible (she also had a non-collapsible cane), so, she testified, she probably had it in her bag. She would take the cane with her in case she had to walk a long distance. The videotape showed her placing the stroller in the back of the vehicle; she did not have any problems doing this.

The Respondent testified that she had six children, four of whom lived with her. On January 30, 2007, she believed her pass was “until 6:00 or 4:00.” She admitted that she “was seen coming after and it’s possible.” She said, “I know I always called if I was coming late, but with the kid and everything that is possible, that I came late.” Two of the children were one-year-old twins. She needed to take them to the doctor, or to go shopping. This time period was right after the Respondent’s father died, so she “was still trying to take care of that business also.” Sometimes she would pick up her five-year-old child from school, and did not have problems doing so. She was also able to carry a “medium-size shopping bag,” as seen on the tape, from the car to her home.

The Respondent explained that in the portion of the tape where she was seen going to the Bronx Health Care Unit, she normally had to park “a block if you’re lucky – a block or two.” The tape showed her using the cane because she had to walk from the vehicle to the 50 Precinct, and she needed the cane for the steps in that building.

The Respondent asserted that she did not try to limp. When she used the cane, she was distributing her weight to the cane, so it might appear that she was limping. She stated that there was no way for her “to just walk regularly with the cane.”

The Respondent testified that the distance from her apartment door to the elevator was less than the distance from the witness stand and the back wall of the Trial Room.⁴

The Respondent testified that she was using crutches in the Trial Room because she was not supposed to put weight on the knee, as the bone was healing. She was currently visiting the

⁴ The Trial Room A diagram listed a lengthwise distance of 47 feet and 7 inches. The Advocate stated that he had “no problem with” the Court’s “estimation of 44 feet” between the witness stand and back wall, and added, “It doesn’t make much of a difference.”

District Surgeon every two weeks. She added, however, "Sometimes two weeks; one week depending on the prior two or last surgery; sometimes every month you'll come back."

On cross-examination, the Respondent agreed that she told Hengel in her official Department interview that she did not use the cane because she lived "in a bad neighborhood and did not want to mark [her]self as an easy target." She added that she worked "midnights" and was out late, and was unarmed because her weapon had been removed due to her injury.

The Respondent testified that after she left the garage in her building, there was a gate and a wall that she could hold onto. The Respondent asserted that on the video, when she entered her vehicle and put her children and the stroller inside, she stepped onto the running board but also grabbed onto the handle above the door attached to the vehicle's frame, and used this to pull herself up. To do this, she held the child with one hand. When she put the child into the car seat, she rested both knees on the regular seat. She asserted that her "right knee is leaning" and her "left knee is bent." She stated that she had no problems opening the vehicle's rear door, folding the twins' double stroller, collapsing it, lifting it, and putting it in the rear storage area, "because the stroller, you push a button and it collapses."

The Respondent testified that she used a cane to walk from her vehicle to the 50 Precinct, rather than hold onto the woman seen next to her – her daughter – because her daughter was at least 100 pounds lighter, and the Respondent thought her daughter would fall also. She used the steps inside the precinct, and needed a cane because her knee would buckle when she flexed it to climb. The Respondent denied that when she picked up the stroller earlier, she also needed to flex her knee.

The Respondent asserted that the elevator at the 50 Precinct had "always been broken," and that the doctor at the Medical Division "would normally come downstairs." She stated that she parked a block away from the precinct on the day recorded in the videotape.

The Respondent clarified, in questioning by the Court, that she used the cane for pain that would arise when she put “[a] lot of pressure” on the knee, “standing, walking.” She stated that the knee “would swell up on me.” She testified that when Mignola spoke to her on January 17, 2007, he did not tell her anything about where she would be assigned. She was technically assigned to the Bronx Court Section during her sick leave, and in fact returned there.

FINDINGS AND ANALYSIS

Specification No. 1

In the first specification, the Respondent is charged with being out of residence while sick, without permission, on January 30, 2007, from 4:00 p.m. to 6:25 p.m. Hengel, the Absence Control investigator, stated that the Respondent was observed “arriving back.” Hengel testified that the Respondent’s out-of-residence pass for that day was between 12:00 p.m. to 4:00 p.m. He added that she did not “have permission or authority to be outside of those hours.”

The Respondent conceded in her testimony that she was observed on the videotape for January 30, 2007, “coming after and it’s possible.” She said, “I know I always called if I was coming late, but with the kid and everything that is possible, that I came late.” On summation, her counsel stated, “If they had someone say that her pass had expired, we’re not really in a position to do anything about it.” He added that “it looks like she stayed out a couple hours later or two hours and 25 minutes.” Counsel stated that the Respondent’s testimony constituted mitigation, but “obviously, there’s been enough to find her guilty of Charge Number One.” This Court concurs and finds the Respondent Guilty of Specification 1.

Specification No. 2

The second specification charges that the Respondent, on or about January 17, 2007,

made false or misleading statements to Medical District personnel exaggerating the extent of her injuries, to wit: Officer Moore told Medical District personnel, in sum and substance, that she could not perform in a restricted duty capacity, and appeared before the Medical District using a cane, which, in light of observations made of Officer Moore, are in fact false.

The Department did not dispute that the Respondent suffered a genuine line-of-duty injury and that her surgeries were necessary. Rather, the Department alleged that the Respondent was exaggerating her pain and injury so as to continue staying out of work on sick leave.

The specification essentially contains two separate allegations: (a) that the Respondent exaggerated her pain to Mignola, the District Surgeon, when she told him on January 17, 2007, that she could not perform even restricted duty; and (b) that she did not truly need a cane to walk when she went to the appointment and brought it to make her situation look worse.⁵

The Department argued that the Respondent was exaggerating when she informed Mignola that she could not perform restricted duty. Her attorney asserted that she was simply being honest. The Respondent testified that it was painful for her to stand. The Respondent testified that it was painful for her to walk as well, and that the pain increased as she walked. She stated that she used a cane when she had to walk distances longer than half a block. The Respondent also testified that during the period around the January 17, 2007, examination, she would need to pick up her children from school, go shopping, and take her children to the doctor. She was also attending to the affairs of her recently-deceased father.

Mignola testified that at the January 17, 2007, examination, the Respondent complained of “ongoing stiffness and pain in the knee.” He also testified that the Respondent said she “was unable to ambulate or sit for any prolonged period of time.” She was “still ambulating with a cane” at this time. Mignola testified that he felt she should have been ready to return by this time

⁵ Although the specification is awkwardly worded, this reflects its obvious meaning.

to restricted duty. It was this suspicion that led Mignola to refer the Respondent to Absence Control.

The Department presented a videotape of observations made of the Respondent (DX-1), and an analysis of the tape will be useful here. The tape began on January 30, 2007, at 2:02.31 p.m. It showed the Respondent bending over, picking up, carrying and placing each of two small children into her SUV, which was parked behind her apartment building. She stepped onto the SUV's running board in order to do so, using her right (uninjured) leg. She walked the children's double stroller toward the rear of the SUV, collapsed the stroller, lifted it to about chest level, and placed it into the back of the car. The Respondent was able to collapse the stroller by a push-button mechanism, as she testified, and did not need to bend to do so. At 2:05 p.m., she stepped up onto the running board again to get into the driver's seat. At no time did the Respondent hold or use a cane for support.

At 6:29.20 p.m., the Respondent's SUV was observed back in the apartment building's parking lot. She got out of the vehicle, and went to the rear. She placed something in the stroller, then walked back to the rear of the vehicle. An adult male and an older child appeared on the tape as well. At 6:33.03 p.m., the Respondent and these two individuals walked away from the vehicle and out of view. The Respondent was not holding a cane, but appeared to be very slightly limping.

The next observations on the tape took place on March 2, 2007. At 11:47 a.m., the Respondent's SUV was observed parked outside a private doctor's office. At 1:08 p.m., she got into her SUV, again using her right leg on the running board. She was not holding a cane. (Hengel testified that the Respondent returned home to pick up another female. The Respondent later identified this second female as her daughter.). At 1:47 p.m., the Respondent parked the vehicle near the Bronx Health Care Unit. At 2:03 p.m., she and her daughter (who appeared to

be in her teens) got out of the vehicle and began walking. The Respondent was holding the cane in her hand. She walked on the sidewalk, next to her daughter, holding the cane in her hand. She can be seen limping slightly, then her limp seemed to get worse. The two then approached the 50 Precinct Stationhouse. This part of the tape ends at 2:04.14 p.m.

At 3:32.57 p.m., the Respondent left the 50 Precinct with her daughter. She still was holding the cane as she walked, and her limp was the same as when she entered the precinct. At 3:34.34 p.m., the Respondent got back into her vehicle, again stepping onto the running board with her right leg.

At 3:55 p.m., the tape showed the Respondent returning to her residence's parking lot. When she got out of the car at 3:58.22 p.m., she appeared to be walking normally, then began limping. She was not holding a cane. As she and her daughter walked down a walkway on the side of the apartment building, the Respondent was still limping, but much less so than when she first got out of the vehicle. They disappeared from view, still walking, at 3:59.23 p.m.

The Department presented the testimony of two Medical Division physicians with regard to knee pain in general. Dr. Lichtenstein testified that in knee cases like that of the Respondent, the pain commences after the patient stands up, but actually lessens as the patient walks. This is because the knee joint is "cold" when the patient is sitting and the knee is flexed, or bent. When the knee moves and blood flows over the joint, the pain decreases. No medical evidence was presented to contradict Lichtenstein's assessment. In fact, when the Respondent's counsel pressed Lichtenstein, "But that's not always the case with knee injuries?," the doctor answered, "That's kind of pretty much always the case." Lichtenstein's colleague, Mignola, agreed with counsel that sitting, standing or running could all cause pain. He also stated that there were knee injuries where prolonged use could cause pain.

The Department also presented evidence of an alleged motive on the Respondent's part to stay out on sick leave longer than necessary. The Respondent had six children, four of whom lived with her, one of whom was five years old, and two of whom were stroller-age twins. Her father had just passed away and she was tending to his affairs. She also mentioned a sister who had medical issues; the Respondent was helping her as well. The Department argued that it would have been easier for the Respondent to do all these things while on sick leave than if she were working, for example, midnight tours at Criminal Court.

As a result of this evidence, the Court makes the following findings. The videotape demonstrated that the Respondent was able to walk for extended periods of time, unassisted by a cane. This contradicts her statement to Mignola that not only could she not perform restricted duty, but that she could not walk or sit for extended periods. If the Respondent could walk as she did on the tape – placing her children into her SUV, putting the stroller in the rear of the vehicle, and walking back from the parking lot to her apartment building – she could perform restricted duty. Nothing was required of her on restricted status beyond what she was doing while she was on sick leave. If the Respondent, while on sick leave, was able to shop, go to doctor's appointments, and pick up her children from school, she was also able to take paperwork from one part of the Bronx Criminal Court building to another.

There are further reasons for caution in accepting the Respondent's claim that she could not walk long distances because the pain increased as she walked. Lichtenstein testified that the opposite almost always was the case: a patient's knee pain is most intense when she is sitting and the knee is bent. When the patient begins to walk, blood flows over the joint and the pain decreases.

The Department also presented the videotape to argue that the Respondent's use of the cane on January 17, 2007, was an exercise of exaggerating. The gist of the Department's

argument was that she used the cane in the vicinity of the 50 Precinct Stationhouse on both days because she knew she would be observed by members of the Department, and that she did so in an attempt to exaggerate her injury.

The videotape evidence demonstrated that Hengel observed the Respondent using the cane only on two occasions: walking to the Bronx Health Care Unit on March 2, 2007, and returning to her vehicle from that doctor's visit. During the remainder of his observations, he never saw her using the cane. On the tape, the Respondent was seen on January 30, 2007, loading her children, their double stroller, and other items in and out of her SUV. This was done without the cane. She was twice observed, once on the January date and once on March 2, 2007, walking to her apartment building from her vehicle without the cane.

Mignola testified that when the Respondent appeared for the January 17, 2007, examination, her limp appeared slightly exaggerated. He also felt that, based on the date of her second surgery, she should have been ready for restricted duty by then. As noted supra, it was the Respondent's testimony that she used the cane when she had to stand for long periods of time, or was going to walk long distances. Lichtenstein testified that at the March 22, 2007, visit, the Respondent also presented with a cane and was limping. She told Lichtenstein that she could walk only about half a block before experiencing "extreme pain."

Mignola also testified about his experience with patients that use canes. He stated that he would expect someone who could not walk more than half a block without pain, as the Respondent characterized her situation, to have to use a cane or other "walking device" at least "most of the time," if not all the time, if the patient used the cane at all.

Based on this evidence, the Court makes the following conclusions. The Court finds that the Respondent walked with the cane to, from and at the Bronx Health Care Unit visit on January 17, 2007, because she was trying to exaggerate the amount of pain that she was in. The

Respondent's assertion that she only used the cane when she knew she had to walk more than "half a block" is not persuasive in light of the physicians' testimony and the videotape. As noted supra, her claim that her pain would increase with more walking was contradicted by Lichtenstein. Also, Mignola testified that a person who used a cane for longer walks would be expected to use it at least most of the time. Thus, it does not make sense that the Respondent would have had to use the cane when she walked to the 50 Precinct, but not at the other times she was observed walking on the videotape. The only times she was seen, either on tape or in person, walking with a cane, was at Medical Division examinations. The Court concludes, therefore, that the Respondent used the cane at the January 17, 2007, examination in order to exaggerate her injury.

In sum, the Department proved that the Respondent was able to walk, bend over, and carry objects as heavy as a 1 year old child and a double stroller. She was thus able to perform restricted duty. Her statement to Mignola that she would be unable to perform restricted duty was therefore an exaggeration, as was using a cane to try and make her injury appear worse. As such, the Respondent is found Guilty of Specification 2.

PENALTY

In order to determine an appropriate penalty, the Respondent's service record was examined. See Matter of Pell v. Board of Education, 34 N.Y.2d 222, 240 (1974). The Respondent was appointed to the Department on April 15, 1997. Information from her personnel folder that was considered in making this penalty recommendation is contained in the attached confidential memorandum.

The Respondent has been found Guilty of exaggerating the nature of her pain and injury so as not to return to restricted duty. She has also been found Guilty of being out of her

residence while on sick leave. As a penalty, the Department argued for the forfeiture of 30 vacation days and the imposition of one year of dismissal probation.

In *Disciplinary Case No. 80050/04*, a Police Officer was found Guilty of exaggerating his injury by stating to a Department physician that he was unable to perform various tasks, like carrying bags or shoveling snow. In fact, he was able to do these things. As a penalty, he forfeited 45 vacation days and was placed on dismissal probation.

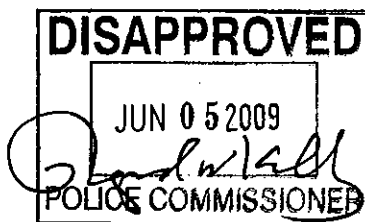
The Department has an interest and responsibility to insure appropriate use of its generous sick leave benefits. The Respondent has abused these benefits, essentially avoiding her job responsibilities in an effort to stay out on sick leave and being dishonest to the Medical Division when she presented herself with a cane. In her favor are her favorable work history (see Confidential Memorandum, *infra*) and the fact that she suffered the relevant line-of-duty injury while subduing an armed suspect.

Thus, the Court recommends that the Respondent be DISMISSED from the New York City Police Department, but that her dismissal be held in abeyance for a period of one year, pursuant to Section 14-115 (d) of the Administrative Code of the City of New York, during which time she is to remain on the force at the Police Commissioner's discretion and may be terminated at any time without further proceedings. The Court further recommends that the Respondent forfeit 30 vacation days.

Respectfully submitted,



David S. Weisel
Assistant Deputy Commissioner – Trials



POLICE DEPARTMENT
CITY OF NEW YORK

From: Assistant Deputy Commissioner – Trials
To: Police Commissioner
Subject: CONFIDENTIAL MEMORANDUM
POLICE OFFICER DELVIA MOORE
TAX REGISTRY NO. 918682
DISCIPLINARY CASE NO. 83467/07

In 2007 and 2005, the Respondent received an overall rating of 3.0 “Competent” on her annual performance evaluation. In 2006, she received a rating of 3.5 “Highly Competent/Competent.” [REDACTED]

[REDACTED] In January 2008, the Respondent was placed on Level-II Discipline Monitoring. She has no prior formal disciplinary record.

For your consideration.



David S. Weisel
Assistant Deputy Commissioner – Trials